

BUSINESS IMPACT · SCALABILITY · PRODUCT EXPANSION

Sheru

From local focus coach to a
privacy-first focus & mental-wellbeing
SaaS platform

Business plan covering monthly psychologist care (Sheru Mind),
SaaS subscriptions, five-year financial model and scaling options.



\$38.1M

2031 exit ARR (base)

263K

paying seats 2031

2030

EBITDA break-even

\$0

AI inference cost / user

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OVERVIEW

1 Executive Summary

Sheru today is a working, privacy-first focus coach. A Chrome MV3 extension classifies every tab against the user's goal using a **local** LLM (Ollama), nudges them in real time, and the Pulse backend turns ActivityWatch data into focus analytics and AI stand-ups. This plan turns it into a business on three layers:

- **SaaS subscriptions.** A free local tier stays free forever. Pro, Teams and Enterprise tiers add opt-in end-to-end-encrypted sync, a hosted dashboard, integrations and admin tools.
- **Sheru Mind.** A monthly subscription that pairs each member with a licensed psychologist. It includes one 50-minute video session a month, async check-ins and a consent-based *Focus Report*, so therapy is grounded in real attention data instead of memory.
- **Enterprise Wellbeing.** Employers buy Sheru + pooled psychologist access as a focus and burnout benefit. They see only anonymised aggregates, never individual browsing.

\$27.1M 2031 revenue (base)	\$38.1M 2031 exit ARR	263K paying seats, 2031	63% 2031 gross margin	2030 EBITDA-positive
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Why this works

- **Structural cost advantage.** Classification runs on the user's own machine, so Sheru pays nothing for AI inference. Cloud-AI rivals pay roughly \$1 per active user per month (Section 10). This keeps software gross margin near 90% and lets the free tier scale to 1.6M users cheaply.
- **Trust is the product.** In mental health and workplace tools, privacy is the purchase criterion. Sheru can truthfully say raw browsing never leaves the device.
- **Data-informed care is differentiated.** Few therapy platforms see objective attention patterns. Sheru Mind gives psychologists a weekly picture of focus, context switching and late-night work, and turns their advice into in-browser nudges.
- **Land-and-expand.** Individual users bring the product into teams. Teams graduate to Enterprise Wellbeing. By 2031, 53% of revenue is B2B.

The ask, in one line

About \$1.6M of cumulative funding (plus a 30–50% buffer) takes the base case to EBITDA break-even in 2030. The first milestone is a 10-psychologist Mind pilot in India in Q2–Q3 2027, used to validate retention and willingness to pay before scaling.

STARTING POINT

2 Where Sheru Stands Today

The repository already contains a functional, tested MVP across three apps. That matters commercially: most of the hard, differentiating work (local classification, nudges, analytics) is done. The work ahead is mostly the commercial shell around it.

Table 2.1 — Product inventory vs. commercial gaps

Component	What exists in the repo	Gap to commercial SaaS
Chrome extension (MV3)	Tab tracking, Ollama classification, 0–100 focus score, DOM nudges, voice nudges, side panel, AI tab grouping, Context Reset, offscreen keep-alive	Licence check, account login, tier feature-flags, Web Store listing, telemetry opt-in
Pulse backend (FastAPI)	Summary, timeline and stand-up APIs over ActivityWatch; graceful fallbacks (sample data, template stand-ups)	Multi-tenant cloud variant, auth, sync ingest, Focus Report generator, billing webhooks
Pulse dashboard (React)	Metric cards, timeline, category breakdown, typewriter stand-up generator, tests	Hosted version, team views, psychologist view, subscription settings
Ops	setup/start/stop/test scripts, docker-compose, three test suites	CI/CD, cloud infra-as-code, monitoring, SOC 2 controls

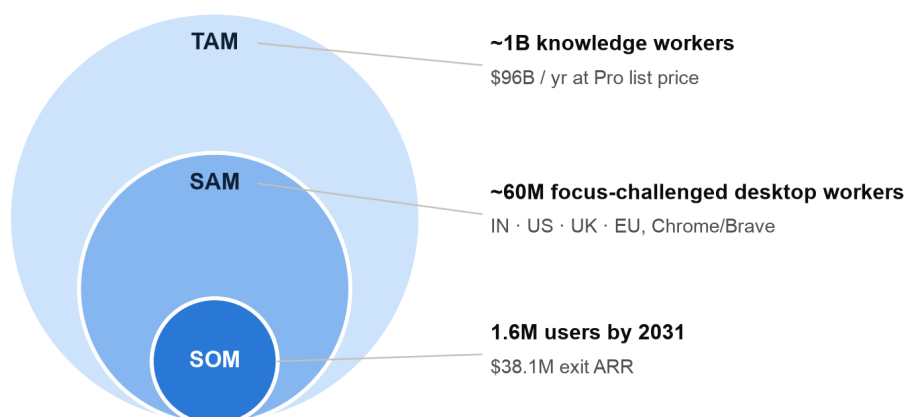
MARKET

3 Problem & Market Opportunity

The problem, sharpened

- **Distraction is now the default state of knowledge work.** Browsers hold most work and most distraction, and static blocklists cannot tell a React tutorial from a cat video on the same site.
- **Attention problems and mental health are linked but treated separately.** The WHO estimates depression and anxiety cost the global economy about **US\$1 trillion a year** in lost productivity. Adult ADHD symptoms affect a notable minority of adults (pooled estimates around 2.5% persistent, higher for symptomatic). Yet productivity apps ignore wellbeing, and therapy apps see no behavioural data.
- **Privacy blocks adoption.** Professionals and employers resist tools that upload browsing history to a vendor cloud, and resistance is even stronger when mental health is involved.

Market sizing



Circles not to scale. Bottom-up sizing; assumptions in Appendix A.

Figure 1 — TAM / SAM / SOM, bottom-up. TAM uses the commonly cited ~1B knowledge-worker estimate multiplied by Pro list price.

Target customers

Table 3.1 — Customer segments

Segment	Who	Primary tier	Buying trigger
ADHD & focus-challenged professionals	Diagnosed or self-identified; often already in therapy or coaching	Mind, Pro	Wants structure without shame; wants therapist to “see” the week
Privacy-conscious developers	Engineers who reject cloud trackers	Free → Pro	Local LLM, open architecture, stand-up generator
Students (exam prep, research)	College & competitive-exam students, especially in India	Pro (student 50% off)	Exam season, parental / mentor encouragement
Remote & hybrid teams	Startups and agencies, 10–200 people	Teams	Meeting overload, deep-work culture, burnout
HR / People / Wellbeing buyers	Mid-market & enterprise, 200+ employees	Enterprise Wellbeing	EAP under-use, burnout metrics, retention
Psychologists (supply side)	RCI-registered (India) or state-licensed (US) clinicians	Partner	Steady client flow, better between-session data, flexible hours

IMPACT

4 Business Impact

Sheru creates value for four parties. The numbers below are the economic case each buyer will test, and the pilot programme (Section 13) is designed to measure them directly.

For individual users

Even modest recovered focus time pays for the subscription many times over. At 25 minutes a day over 220 working days, a user regains about 92 hours a year. Valued at a conservative \$12/hour (India) that is about \$1,100; at \$40/hour it is about \$3,700. Pro costs \$96 a year at full global list price.

Annual value of recovered focus time per user

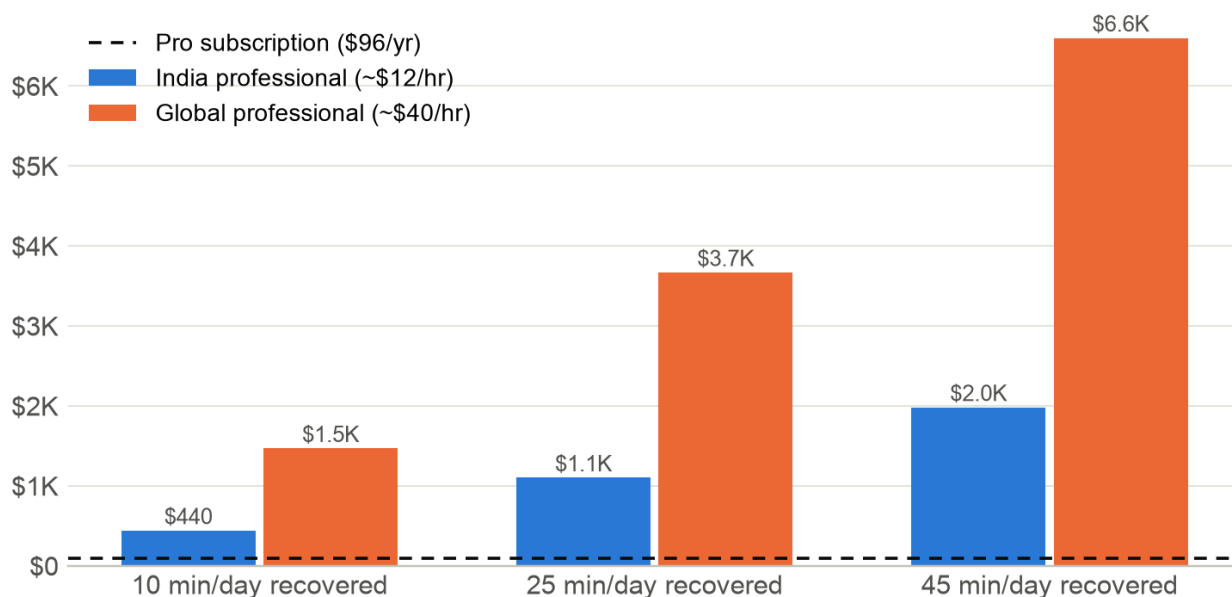


Figure 2 — Annual value of recovered time vs. Pro price. Illustrative; the time-recovered figure will be measured in the pilot.

For employers (Teams & Enterprise Wellbeing)

Table 4.1 — Employer value levers

Impact lever	Mechanism	Illustrative value (100-person team)
Recovered deep-work time	Real-time nudges, calendar focus blocks, Slack DND sync	20 min/day × 220 days × \$30/hr ≈ \$220K vs. \$12K Teams cost (18×)
Faster stand-ups & reporting	AI stand-ups from real activity	~10 min/person/day of status writing removed
Burnout early warning	Anonymised trends in switch rate and after-hours work ($k \geq 5$)	Earlier intervention; lower regretted attrition (to validate)
Higher EAP utilisation	Psychologist access inside a tool people already open daily	Typical EAPs see low single-digit usage; embedded access targets 8%+ monthly

For psychologists

- **Steady demand without marketing.** Matched clients arrive pre-screened, and payouts are predictable (about 50% of the Mind fee).
- **Objective between-session data.** The Focus Report shows patterns clients cannot recall accurately: when focus collapses, which triggers recur, and how sleep-time browsing trends.
- **Homework that actually runs.** Goals set in session become live nudges in the browser. This closes the gap between the therapy room and the workday.

For Sheru as a company

- Mind raises ARPU about 7× over Pro and creates the strongest switching cost: a relationship with a clinician.
- B2B tiers bring low churn (1–1.5% monthly) and annual contracts, which stabilise cash flow.
- Outcome data (self-reported wellbeing and focus trends, anonymised) builds a defensible evidence base for insurer and enterprise sales.

NEW FEATURE

5 Sheru Mind — Monthly Psychologist Care

Sheru Mind is the flagship new feature. Members get a monthly relationship with a matched psychologist who specialises in attention, ADHD, burnout or performance anxiety. The extension becomes the bridge between sessions.

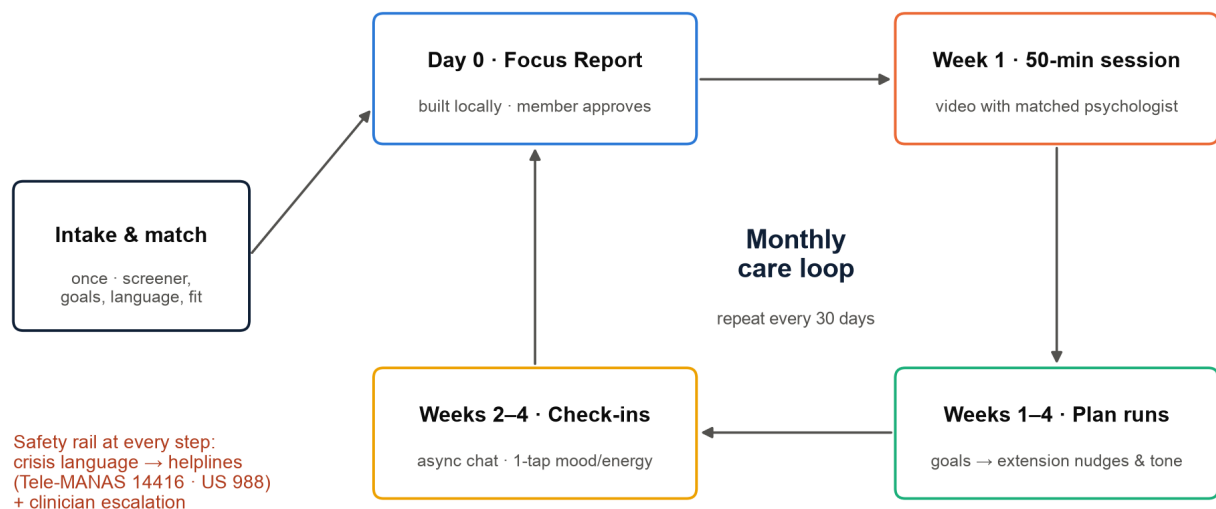


Figure 3 — The Sheru Mind monthly care loop.

What a member gets each month

Table 5.1 — Mind membership contents

Component	Detail
1 × 50-minute video session	Scheduled in-app with the matched psychologist. Reschedule up to 24h before. Extra sessions available as an add-on.
Focus Report (consent-gated)	Generated locally from ActivityWatch + focus scores: weekly focus trend, distraction hot-hours, context-switch rate, session completion, mood/energy check-ins. The member previews it and chooses what to share. Raw URLs are never included by default.
Async check-ins	Secure messaging with the psychologist; response within 2 business days. Weekly structured check-in prompts.
Plan → nudges	Goals and coping strategies set in session flow into the extension. Examples: nudge tone (gentle vs. firm), deep-work block length, wind-down reminders, break suggestions.
Daily 1-tap mood & energy	In the side panel; feeds the Focus Report and burnout detection.
Progress measures	Monthly WHO-5 Well-Being Index and ASRS-based self-report (non-diagnostic), shown to member and psychologist.

Psychologist portal

- Client roster with pre-session brief (shared Focus Report and last check-ins).
- Encrypted session notes (clinician-owned), goal-setting UI that pushes to the client's extension.
- Calendar and availability, video room, async inbox, payouts and earnings dashboard.
- Caseload cap and utilisation view, so quality is protected and no clinician is overbooked.

Matching, credentialing and safety

Table 5.2 — Clinical governance

Area	Policy
Credentialing	India: RCI-registered clinical psychologists, or counselling psychologists with recognised postgraduate qualifications, verified on onboarding. US/EU (2028+): licensed in the member's state/country. Annual re-verification and peer supervision.
Matching	Intake questionnaire (goals, ASRS v1.1 screener, language, timezone, preferences). The algorithm proposes 3 clinicians and the member chooses. A free switch is available any time.
Scope	Mind is psychological support and coaching for focus and wellbeing. It is not an emergency service and does not diagnose or prescribe. Psychiatric needs are referred out.
Crisis protocol	Crisis language in check-ins triggers immediate in-app helplines (India Tele-MANAS 14416; US 988; local equivalents) and a same-day clinician escalation. Protocol is reviewed by a clinical advisory board.
Quality	Session ratings, outcome trends, random case audits, clinician NPS, and a cap of about 80 members per full-time psychologist.

Why monthly, not weekly?

Weekly therapy costs 4× more and suits acute needs. Mind targets sub-clinical, persistent focus and burnout problems. There, a monthly session plus a daily in-browser coach is a better price-to-value fit. Members who need more can add sessions, and clinicians can recommend a higher-intensity external service.

NEW FEATURES

6 SaaS Platform & Subscription Features

Core SaaS capabilities

Table 6.1 — SaaS feature set by tier

Feature	Description	Tier
Accounts & licensing	Email / Google / passkey sign-in; signed licence token cached in the extension so it works offline	All
Opt-in E2EE sync	Aggregates (focus scores, category minutes, sessions) encrypted on-device; server stores ciphertext only. Multi-device history.	Pro+
Hosted dashboard	The Pulse dashboard as a web app: unlimited history, trends, weekly AI coach report by email	Pro+
Integrations	Google / Outlook calendar focus blocks, Slack status & DND, Jira / Linear / GitHub for richer stand-ups	Pro+
Burnout detection	Switch-rate, after-hours and streak analysis → break suggestions and weekly risk signal	Pro+
Focus rooms	Body-doubling: join a silent co-working room with live focus status	Pro+
Team workspaces	Invites, seat management, shared focus norms, team stand-ups	Teams+
Team insights	Aggregated, anonymised trends with k-anonymity ≥ 5. No individual browsing view, by design.	Teams+
SSO / SCIM / audit log	Okta, Azure AD, Google Workspace provisioning; admin audit trail	Enterprise
Pooled psychologist access	Employer-funded sessions with confidential booking; employer sees only utilisation counts	Enterprise
Desktop & mobile apps	Native focus tracking outside the browser; mobile check-ins & session joining	Pro+

Subscription & billing system

Table 6.2 — Subscription mechanics

Capability	Implementation
Payment rails	Stripe (global cards, Apple/Google Pay) and Razorpay (India: UPI AutoPay mandates, cards, net-banking) behind one internal billing service
Plans	Monthly and annual (2 months free), regional pricing by country, student plan, family plan (up to 4), seat-based Teams, annual Enterprise contracts with invoicing
Lifecycle	Trialing → Active → Past due (smart retries, 3 dunning emails over 10 days) → Grace (read-only) → Cancelled; pause up to 3 months; win-back offers
Changes	Instant upgrades with proration; downgrades at period end; Mind add-on sessions billed per use
Entitlements	Billing events → entitlement service → signed licence token; the extension re-checks daily and degrades gracefully to Free offline
Tax & compliance	GST invoicing (India), VAT/OSS (EU), sales tax via the provider; RBI e-mandate rules for recurring Indian payments
Clinician payouts	Monthly payouts per completed session + check-in stipend; TDS handling (India); payout statements in the portal

MONETISATION

7 Pricing & Packaging

Table 7.1 — Price book

Tier	Global price	India price	Who it's for	Key inclusions
Free	\$0	INR 0	Everyone	Local AI classification, nudges, side panel, 7-day local dashboard. No account needed.
Pro	\$8/mo · \$72/yr	INR 299/mo · INR 2,499/yr	Individuals	Sync, unlimited history, hosted dashboard, AI coach report, integrations, burnout detection, focus rooms
Mind	\$69/mo (US: \$129, 2028)	INR 2,499/mo	Individuals wanting care	Everything in Pro + monthly 50-min psychologist session, async check-ins, Focus Report, mood tracking
Teams	\$10/seat/mo (annual)	INR 399/seat/mo	Teams 5–200	Pro for all + workspace, team insights (k ≥ 5), admin, shared stand-ups
Enterprise Wellbeing	From \$18/employee/mo	Custom	200+ employees	Teams + SSO/SCIM, pooled psychologist sessions, wellbeing reporting, DPA, SLA

Table 7.2 — Add-ons

Add-on	Price
Extra psychologist session (50 min)	\$39 · INR 1,299
Student discount (Pro)	50% with verified student email
Family plan (Pro, up to 4)	\$14/mo · INR 549/mo
Mind for Teams (per enrolled employee)	Volume-priced, from \$35/mo

Pricing rationale

Pro is anchored below mainstream productivity apps so the free-to-paid jump is easy. Mind is priced well below the cost of four monthly therapy sessions, yet high enough for about 50% clinician payout. India pricing reflects purchasing power and lets the clinician network start where qualified supply is affordable. The model uses lower blended ARPUs than list prices to account for regional mix and annual discounts.

TECHNOLOGY

8 SaaS Architecture & Privacy Model

The core principle is **local-first, consent-based sync**. Everything Sheru does today keeps running on the device with no account. The cloud layer is additive and only ever receives what the user explicitly allows, encrypted on-device.

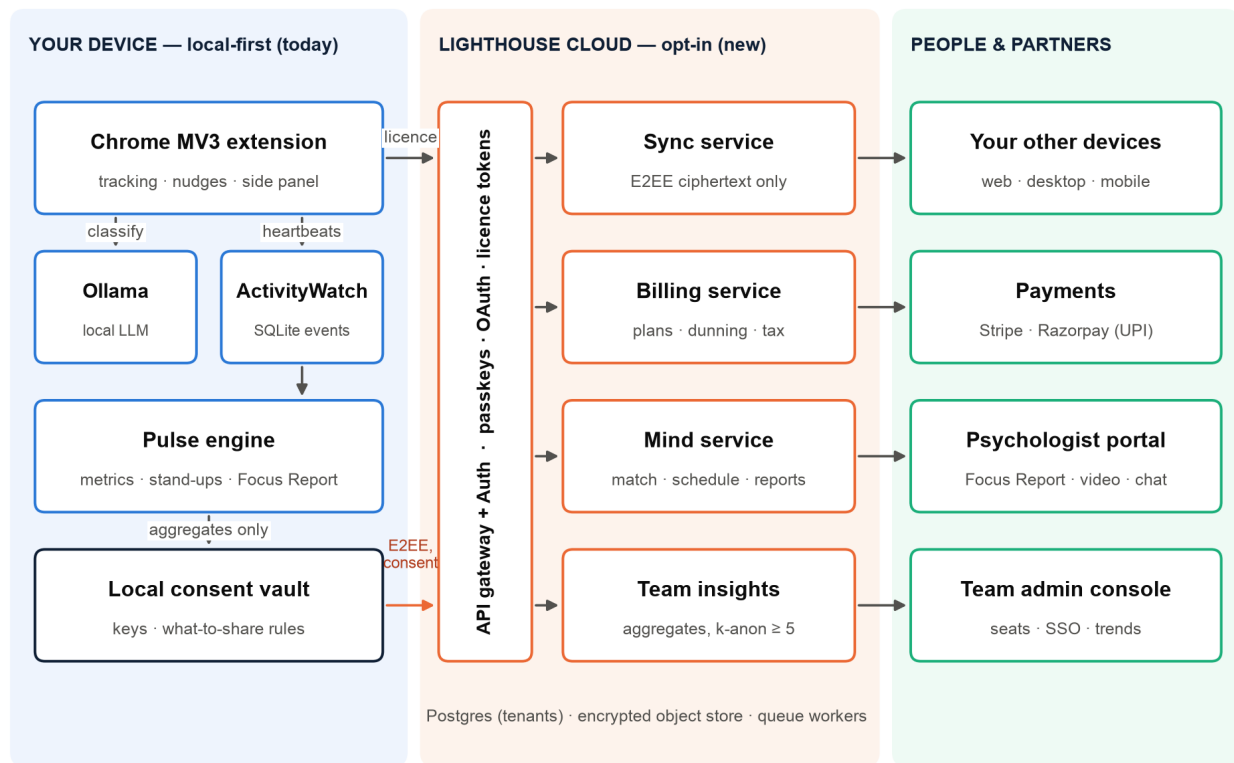


Figure 4 — Target architecture. The left zone is today's repository; centre and right are new.

Table 8.1 — Data classification & visibility

Data class	Stays on device	Syncs (opt-in, E2EE)	Who can read it
Raw URLs & page titles	Always	Never by default	User only
Focus scores & categories (per minute)	Yes	Pro+ sync	User's devices
Daily aggregates & Focus Report	Yes	When shared	User + chosen psychologist
Mood / energy check-ins	Yes	Mind members	User + psychologist
Session notes	—	Clinician-owned, encrypted	Psychologist (user on request)
Team metrics	—	Aggregated server-side	Admins, only if ≥ 5 members
Billing data	—	Held by payment provider	Billing service

FINANCIALS

9 Five-Year Financial Projections

Base-case model, annual granularity, starting with a public launch in early 2027. All figures in USD. Assumptions are listed in Appendix A and in the generator script, so every chart can be re-run with different inputs.

Paying seats at year end, by tier

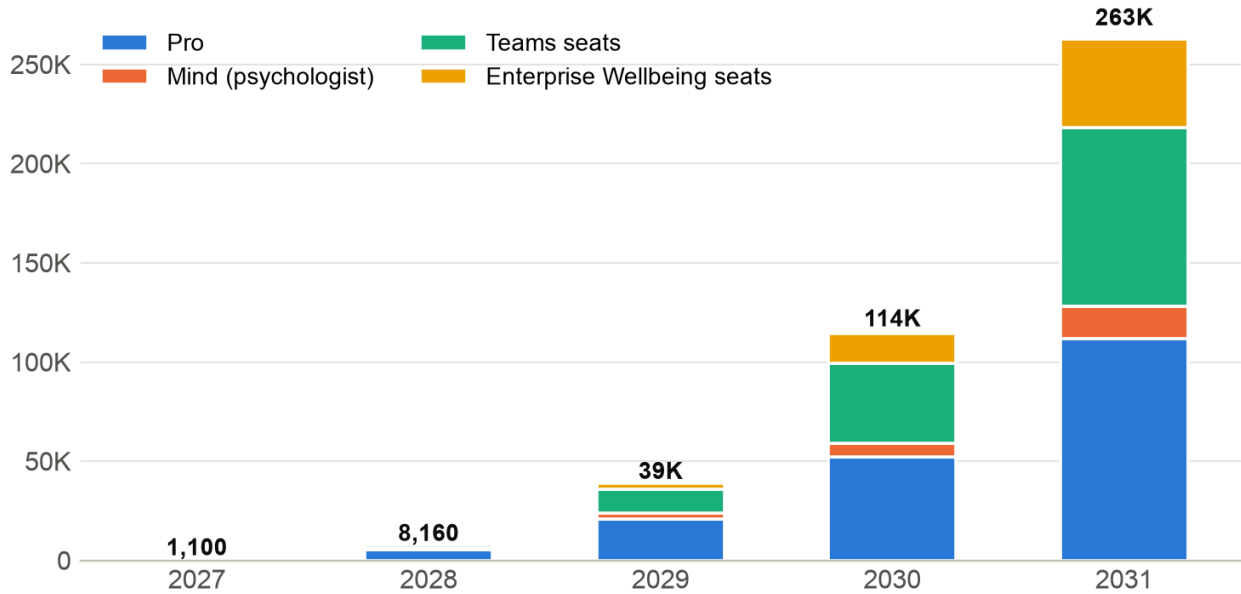


Figure 5 — Paying seats by tier at year end.

Recognised revenue by stream (base case)

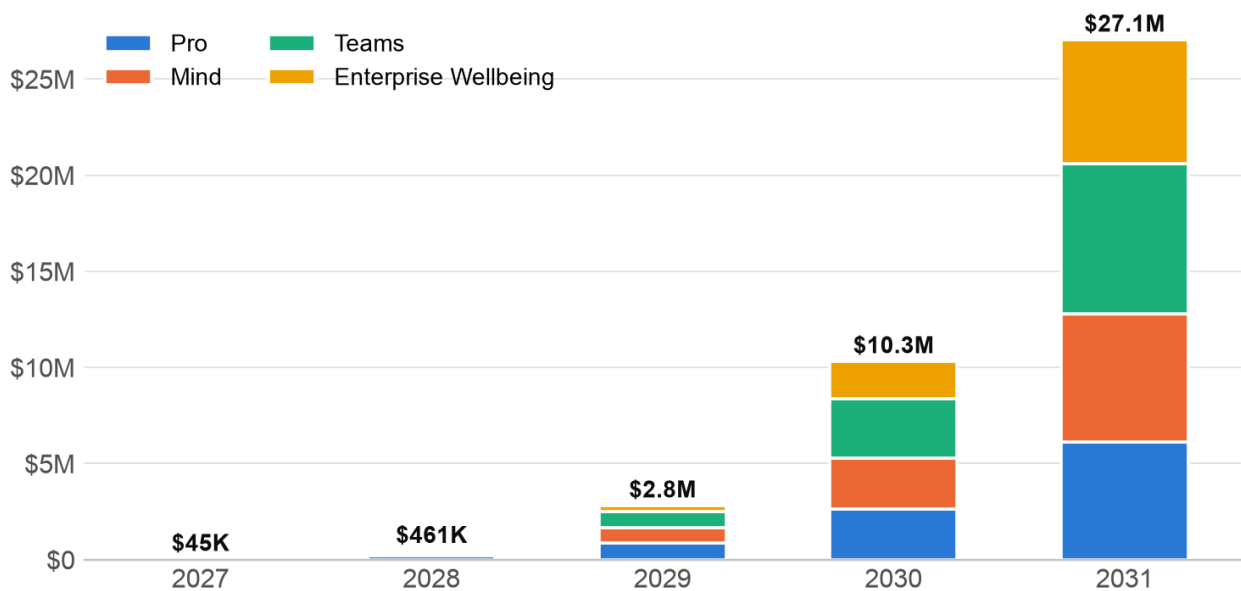


Figure 6 — Revenue by stream.

2031 revenue mix — 53% from B2B (Teams + Enterprise)

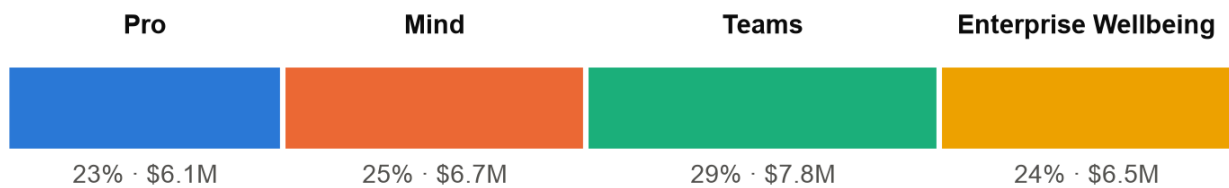


Figure 7 — 2031 revenue mix.

Exit ARR by scenario

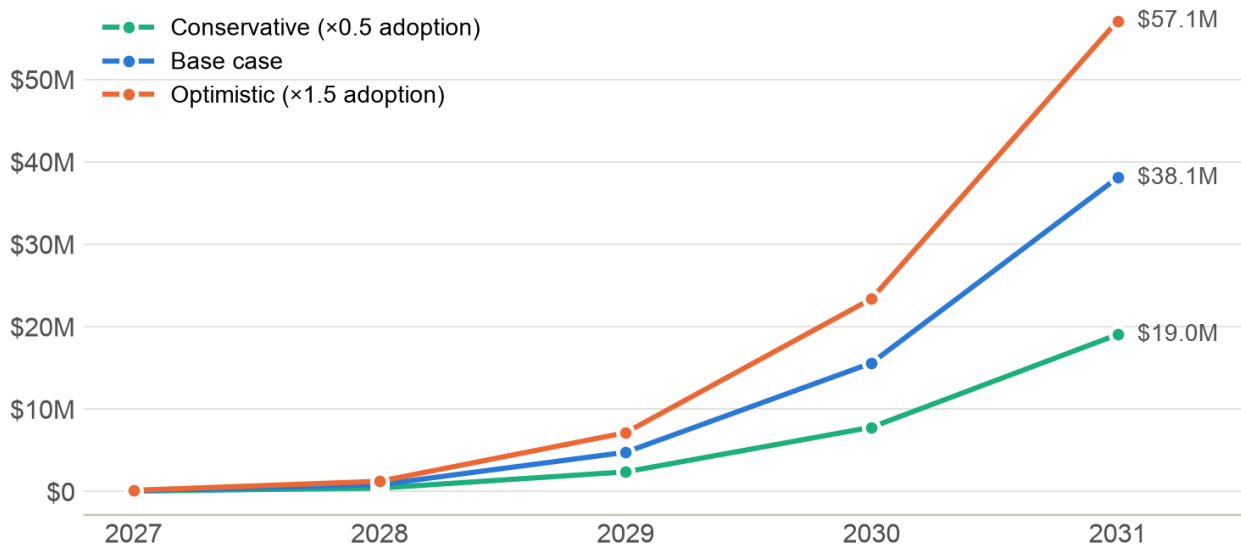


Figure 8 — Exit ARR under three adoption scenarios.

Revenue, cost and EBITDA — EBITDA-positive in 2030

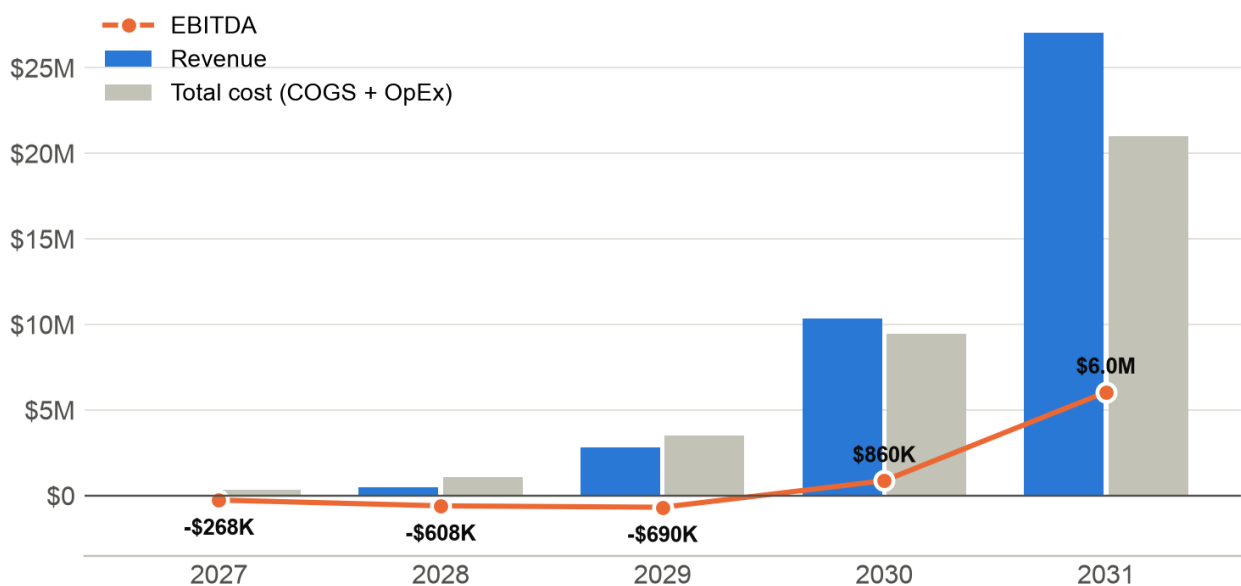


Figure 9 — Revenue, total cost and EBITDA.

Table 9.1 — Summary P&L (base case)

USD	2027	2028	2029	2030	2031
Registered users	25K	110K	350K	800K	1.6M
Paying seats	1,100	8,160	39K	114K	263K
Revenue	\$45K	\$461K	\$2.8M	\$10.3M	\$27.1M
Pro	\$27K	\$195K	\$874K	\$2.6M	\$6.1M
Mind	\$18K	\$146K	\$789K	\$2.6M	\$6.7M
Teams	\$0	\$120K	\$840K	\$3.1M	\$7.8M
Enterprise Wellbeing	\$0	\$0	\$324K	\$1.9M	\$6.5M
Cost of revenue	\$31K	\$217K	\$1.1M	\$3.9M	\$9.9M
of which psychologist payouts	\$9.0K	\$73K	\$492K	\$1.9M	\$5.3M
Gross profit	\$14K	\$243K	\$1.7M	\$6.5M	\$17.1M
Gross margin	32%	53%	60%	63%	63%
Operating expenses	\$283K	\$852K	\$2.4M	\$5.6M	\$11.1M
EBITDA	-\$268K	-\$608K	-\$690K	\$860K	\$6.0M
Cumulative EBITDA	-\$268K	-\$877K	-\$1.6M	-\$706K	\$5.3M
Exit ARR	\$90K	\$823K	\$4.8M	\$15.6M	\$38.1M
Headcount	6	14	30	55	85

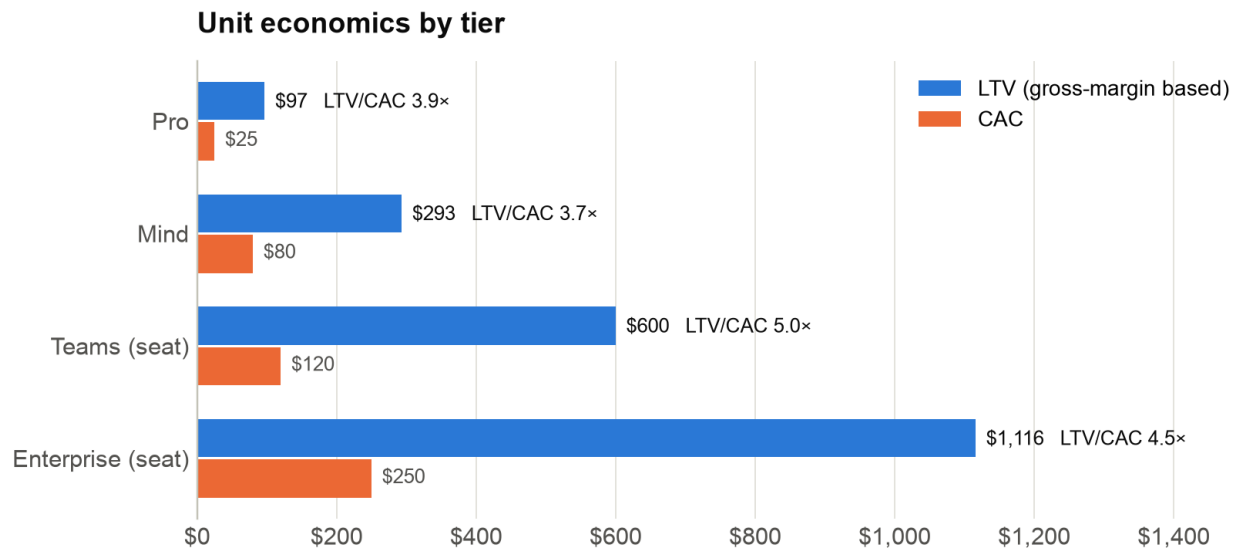


Figure 10 — LTV vs. CAC by tier. $LTV = ARPU \times \text{gross margin} \div \text{monthly churn}$.

Table 9.2 — Unit economics

Tier	ARPU / mo	Gross margin	Monthly churn	CAC	LTV	LTV/CAC	Payback
Pro	\$5.50	88%	5.0%	\$25	\$97	3.9×	5.2 mo
Mind	\$40	44%	6.0%	\$80	\$293	3.7×	4.5 mo
Teams (seat)	\$10	90%	1.5%	\$120	\$600	5.0×	13.3 mo
Enterprise (seat)	\$18	62%	1.0%	\$250	\$1,116	4.5×	22.4 mo

Reading the numbers

Mind has the lowest gross margin (44%) because roughly half the fee goes to the psychologist, but it has the highest absolute LTV among individual tiers. Mind is also the main reason members stay subscribed to Pro features. Teams and Enterprise carry the business long-term: low churn, annual prepayment and LTV/CAC around 5×.

SCALE

10 Scalability Options

Scaling Sheru is unusual. The expensive part of most AI products, inference, does not grow with users here. The constraints that do grow are sync storage, video, clinician supply and compliance. Each needs its own scaling plan.

10.1 Technical scaling stages

Table 10.1 — Infrastructure roadmap

Stage	Scale	Architecture	Key investments
1 · Launch	≤ 10K paid	Single region (Mumbai), managed Postgres, serverless API, hosted video & payments	CI/CD, error tracking, backups, pen-test
2 · Growth	10K–100K paid	Multi-AZ, read replicas, Redis, queue workers for reports, CDN for extension updates	SOC 2 Type II, on-call, feature flags, cost dashboards
3 · Global	100K+ paid / 1M+ users	Multi-region with data residency (IN, EU, US), tenant-sharded Postgres, event streaming, dedicated scheduling service	HIPAA-eligible stack, ISO 27001, regional clinician ops

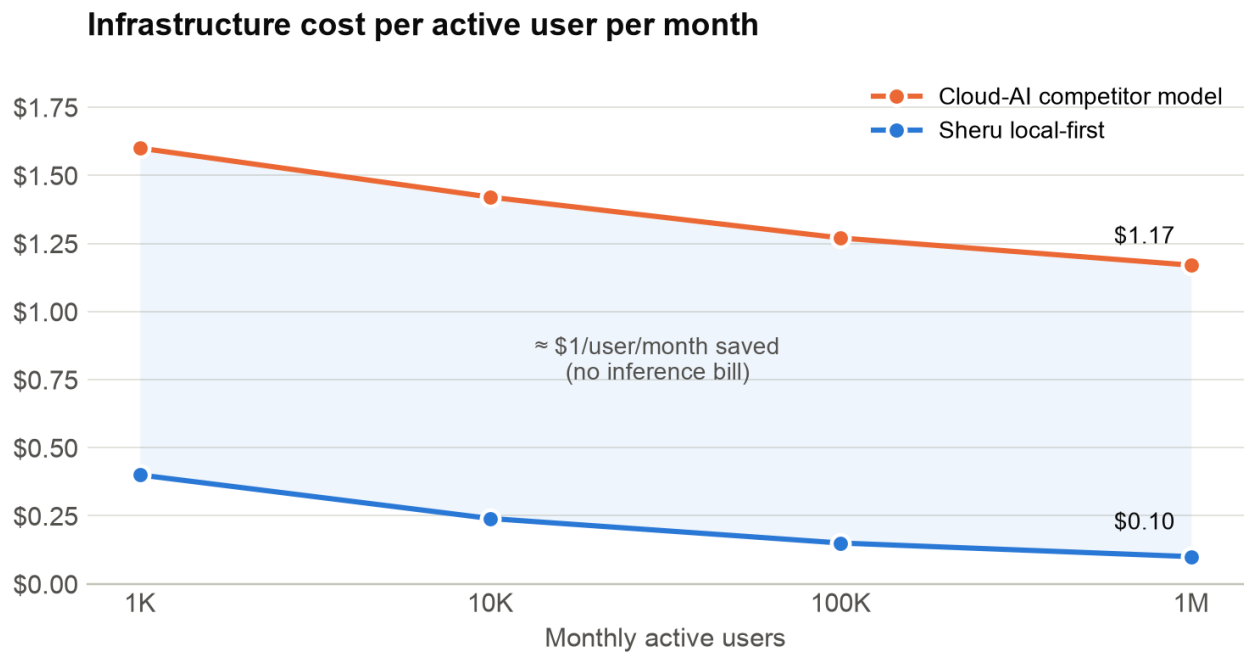


Figure 11 — Per-user infra cost. A cloud-AI model assumes ~400 classifications/user/day × ~300 tokens ≈ 3.6M tokens/month at ~\$0.30 per million tokens.

10.2 Clinician network scaling

Clinician supply is the main operational constraint. One full-time psychologist can carry about 80 Mind members (one session each plus async check-ins). The base case needs about 335 FTE psychologists by 2031, mostly part-time contractors.

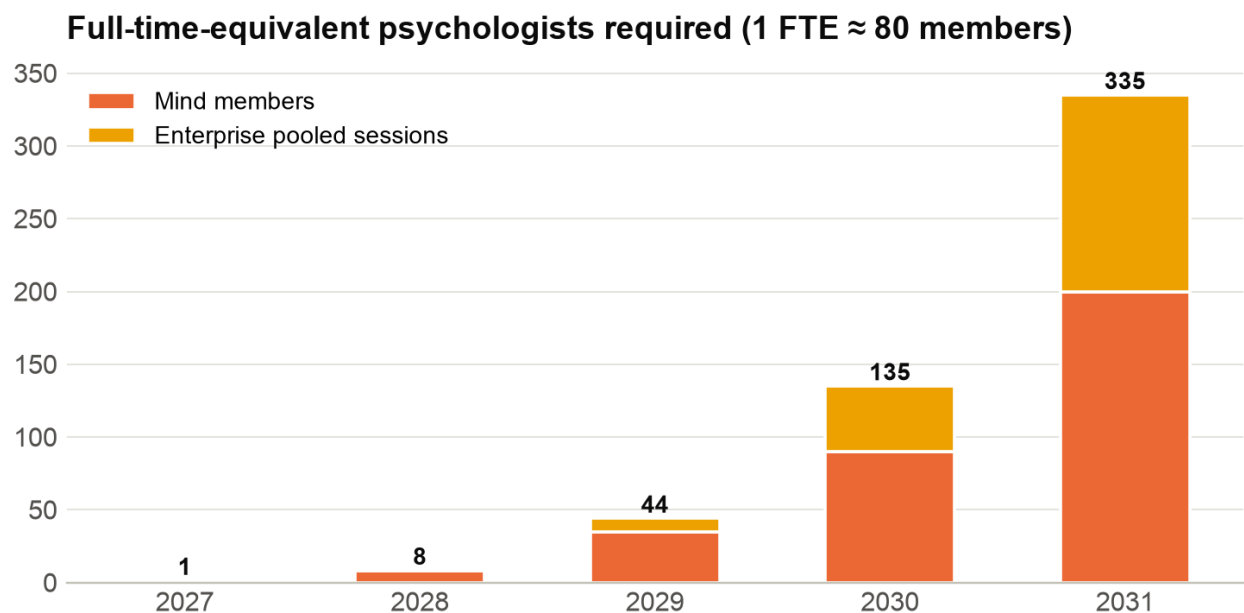


Figure 12 — Psychologist FTE requirement.

- **Supply levers:** university and RCI partnerships, supervised associate tier, part-time contracts, referral bonuses, and a portal that cuts admin time.
- **Efficiency levers:** AI-drafted pre-session briefs from the Focus Report (clinician-reviewed), group sessions for common themes (e.g., ADHD at work), and structured check-in templates.
- **Quality guardrails:** hard caseload caps, supervision ratios, and outcome monitoring before each hiring wave.

10.3 Strategic scaling options

Table 10.2 — Scaling options compared

Option	What it means	Investment	Time to value	Upside	Risk
A · B2C depth (India first)	Grow Free → Pro → Mind in India, then SEA	Low	3–6 mo	Medium	Low ARPU
B · B2B Teams	Bottom-up from individual users to team plans	Medium	6–12 mo	High	Surveillance perception
C · Enterprise Wellbeing / EAP	Sell as a wellbeing benefit, incl. pooled therapy	High	12–18 mo	Very high	Long sales cycles
D · B2B2C via insurers & universities	Distribution through health insurers, campuses	Medium	12–24 mo	Very high	Partner dependency
E · Platform / API	Licence the local focus engine to other apps (on-device SDK)	Medium	18+ mo	High	Focus dilution
F · Geographic expansion	US / UK / EU with locally licensed clinicians	High	12–24 mo	Very high	Regulation, clinician cost

Recommended sequence

A → B → C, with D opened opportunistically. Prove Mind retention in India (A), let users pull Sheru into their teams (B), then turn anonymised outcome data into the Enterprise Wellbeing sale (C). Expand geographically (F) only once clinical operations are repeatable.

GROWTH

11 Go-to-Market

Target acquisition funnel (per 100 store visitors)

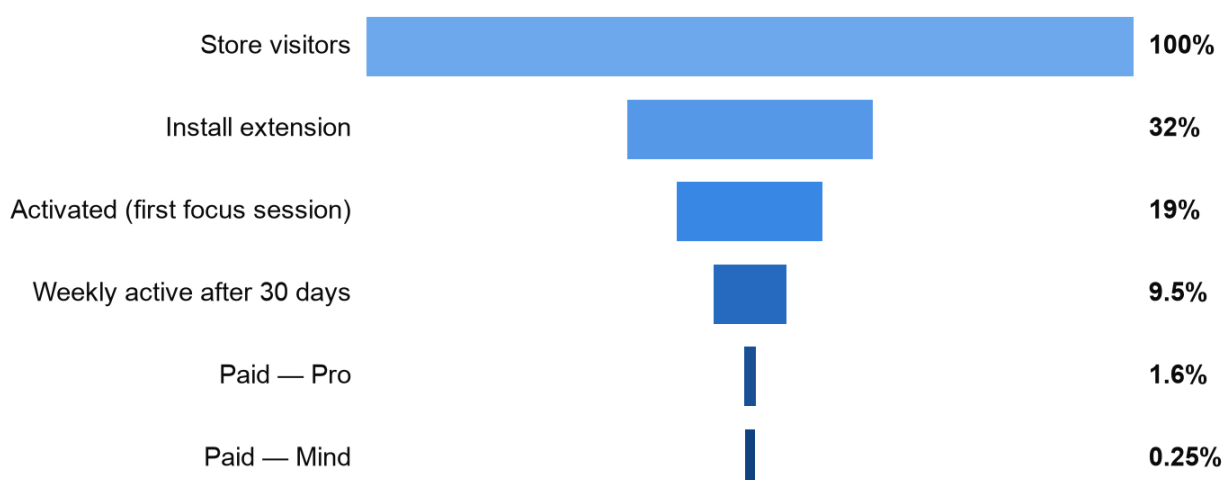


Figure 13 — Target funnel; benchmarks to be replaced with measured data after launch.

Table 11.1 — Acquisition channels

Channel	Tactic	Primary tier
Chrome Web Store & SEO	Store optimisation, “ADHD focus extension”, “privacy time tracker” content	Free → Pro
Developer community	Open-source local engine, GitHub, Hacker News, Product Hunt launch	Free → Pro
ADHD & mental-health creators	Partnerships with creators and communities; psychologist-led webinars	Mind
Psychologist referrals	Clinicians invite existing clients to use Sheru between sessions	Mind
Campus programmes	Student ambassadors, exam-season campaigns, institutional licences	Pro
Product-led B2B	“Invite your team” in-product, team trials, usage-based expansion	Teams
Direct sales & HR channels	HR-tech marketplaces, benefits brokers, wellbeing conferences	Enterprise

GOVERNANCE

12 Compliance, Privacy & Risk

12.1 Regulatory map

Table 12.1 — Compliance obligations (to be confirmed with counsel)

Jurisdiction	Regime	What it requires of Sheru
India	Digital Personal Data Protection Act 2023 (+ Rules)	Notice & consent, purpose limitation, data-principal rights, breach reporting, grievance officer
India	Mental Healthcare Act 2017; RCI norms	Confidentiality of mental-health information; qualified, registered professionals
India	RBI e-mandate rules	Pre-debit notification and authentication for recurring payments
EU / UK	GDPR / UK GDPR	Health data is special-category: explicit consent, DPIA, EU data residency, DPAs with employers
US	HIPAA (if acting with covered entities), state licensure, FTC Health Breach Notification Rule	BAAs, HIPAA-eligible infra, state-licensed clinicians, no deceptive health claims
All	Chrome Web Store policies	Limited-use data disclosure, single purpose, privacy policy

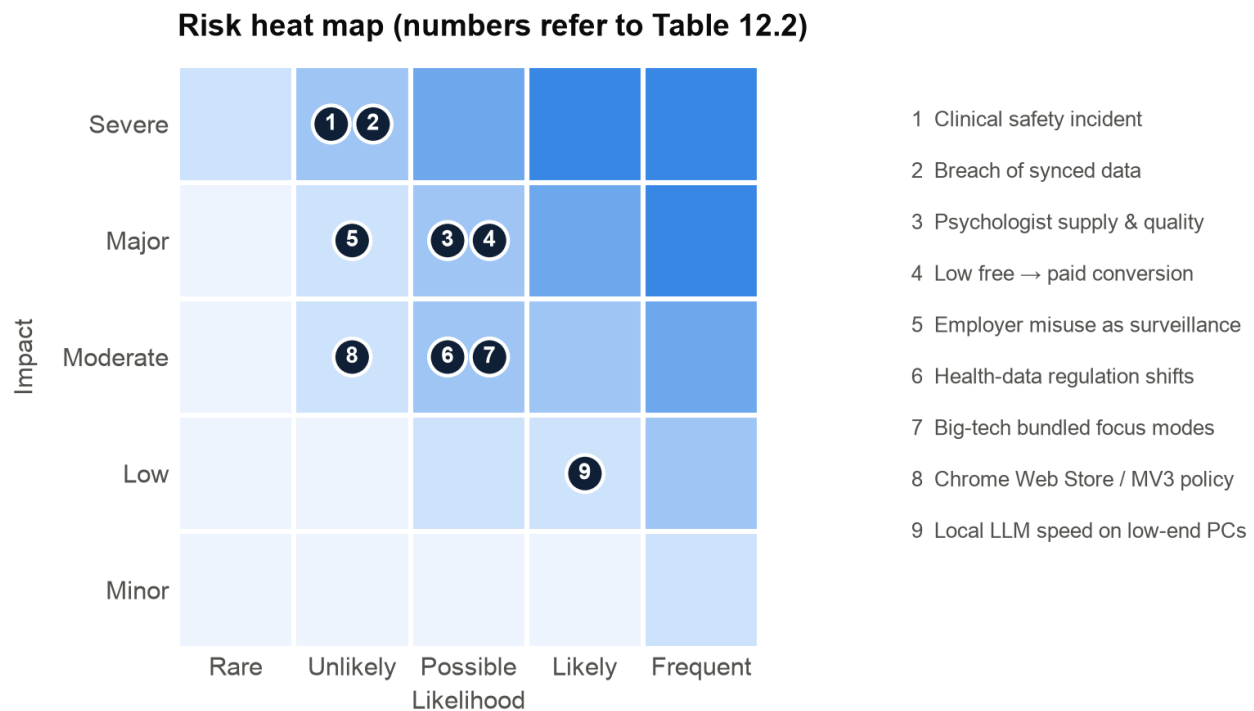


Figure 14 — Risk heat map.

Table 12.2 — Risk register

#	Risk	Mitigation
1	Clinical safety incident	Crisis protocol, helplines, clinical advisory board, insurance, clear scope (not an emergency service)
2	Breach of synced data	E2EE (server holds ciphertext), minimal data, pen-tests, SOC 2, bug bounty
3	Psychologist supply & quality	Multi-channel recruiting, caseload caps, supervision, outcome monitoring
4	Low free → paid conversion	Value-gated features (history, sync, reports), trials, Mind as premium anchor
5	Employer misuse as surveillance	k-anonymity ≥ 5, no individual view by design, contractual restrictions
6	Health-data regulation shifts	Privacy-by-design, regional residency, compliance budget
7	Big-tech bundled focus modes	Differentiate on care + context-aware AI + privacy
8	Chrome Web Store / MV3 policy	Strict policy compliance, desktop app as second surface
9	Local LLM speed on low-end PCs	Smaller quantised models, heuristic fallback (already in repo), optional private cloud inference

EXECUTION

13 Roadmap & Milestones

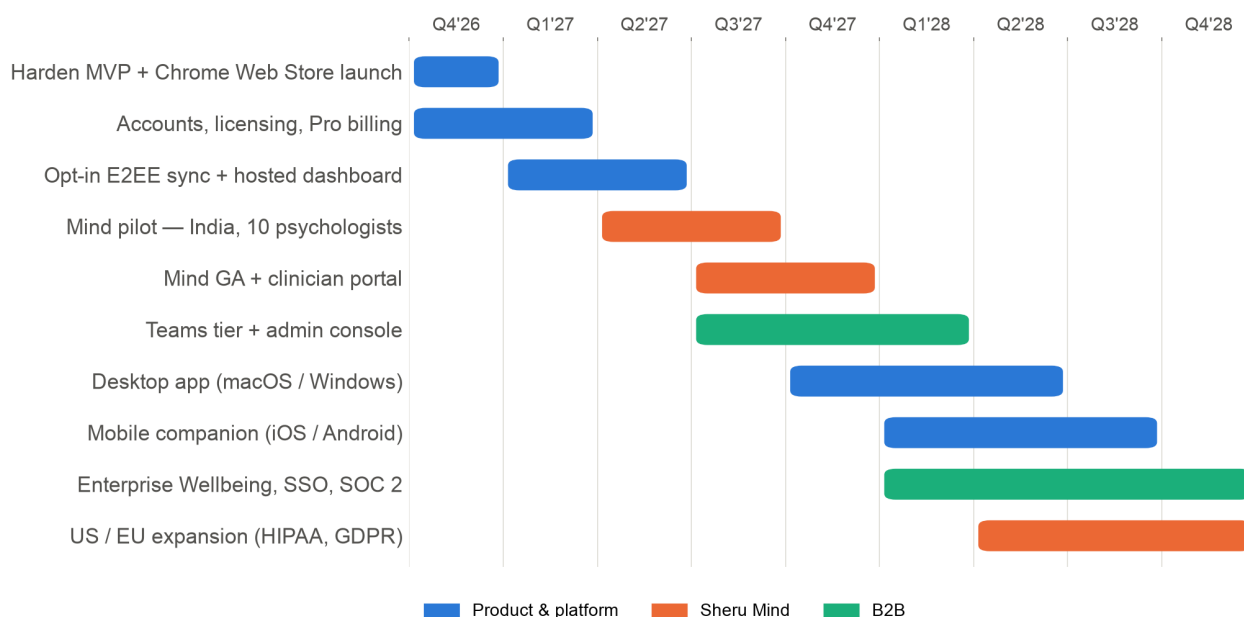


Figure 15 — Execution roadmap, Q4 2026 – Q4 2028.

Table 13.1 — Milestones

Milestone	Target	Success criterion
Web Store launch + Pro billing live	Q1 2027	5K installs; first 100 paying users
Mind pilot (India, 10 psychologists, ~300 members)	Q3 2027	≥ 75% month-3 retention; session attendance ≥ 85%; WHO-5 improvement
Teams GA	Q1 2028	25 paying teams; net revenue retention ≥ 110%
First Enterprise Wellbeing contracts	Q4 2028	3 logos, ≥ 1,000 seats; SOC 2 Type II
EBITDA break-even	2030	Exit ARR ≥ \$15.6M

MEASUREMENT

14 KPIs & Success Metrics

Table 14.1 — KPI tree

Layer	Metric	Target
North star	Weekly focused hours per active user	+20% vs. first-week baseline by week 6
Activation	Install → first focus session within 24h	≥ 60%
Engagement	D30 weekly-active retention	≥ 30%
Monetisation	Free → paid conversion (12 months)	≥ 5%
Revenue	MRR growth · Net revenue retention (B2B)	15% MoM early · ≥ 110%
Mind quality	Session attendance · Session CSAT · Month-3 retention	≥ 85% · ≥ 4.6/5 · ≥ 75%
Outcomes	WHO-5 change · self-reported focus change	Meaningful improvement by month 3
Clinician ops	Utilisation · clinician NPS · time-to-first-session	70–85% · ≥ 50 · < 72h
Trust	Data incidents · % users enabling sync	0 · tracked

APPENDIX

A Appendix — Model Assumptions & Sources

Table A.1 — Model inputs

Assumption	Value	Note
Registered users (year end)	25K · 110K · 350K · 800K · 1.6M	2027 → 2031
Pro share of users	4.0% · 5.0% · 6.0% · 6.5% · 7.0%	Freemium benchmark range
Mind share of users	0.4% · 0.6% · 0.8% · 0.9% · 1.0%	Premium care tier
Teams seats (year end)	0 · 2,000 · 12K · 40K · 90K	From 2028
Enterprise seats (year end)	0 · 0 · 3,000 · 15K · 45K	From 2029
Blended ARPU Pro / Mind	\$4.5→\$6.2 / \$30→\$48	India-heavy early mix
Blended ARPU Teams / Enterprise	\$10 / \$18 per seat	Annual contracts
Psychologist payout	50% of Mind; 30% of Enterprise	Per completed session
Infra cost	\$0.10 free / \$0.60 paid per month; \$0.50 per video session	No inference cost (local LLM)
Payments + support	3% + 4% of revenue	
Headcount	6 · 14 · 30 · 55 · 85	India-based core team
Marketing	\$40K · \$260K · \$900K · \$2.3M · \$4.8M	
Clinician capacity	80 Mind members per FTE	

Sources & notes

- World Health Organization, *Mental health at work* (2022): ~12 billion working days lost each year to depression and anxiety, costing about US\$1 trillion in productivity.
- Gartner's widely cited estimate of ~1 billion knowledge workers worldwide (used for TAM).
- Song P. et al., *Journal of Global Health* (2021): global prevalence of adult ADHD (persistent $\approx 2.6\%$; symptomatic $\approx 6.8\%$).
- WHO-5 Well-Being Index and the WHO Adult ADHD Self-Report Scale (ASRS v1.1) are publicly available screening instruments. Sheru uses them only for non-diagnostic self-tracking.
- Helplines: India Tele-MANAS 14416; US 988 Suicide & Crisis Lifeline.
- Before external use, verify these figures against the latest editions. Financial projections are illustrative planning estimates, not forecasts or guarantees. Regulatory points are a starting checklist, not legal advice.

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